coursera

Anxiety and Related Disorders

Today's goal • 3/3 items

Module 1
Learn With PsycLearn Essentials

Module 2
Introduction to Psychological Disorders

Module 3
Opening to Anxiety Disorders

Module 4
Components of Anxiety

Module 5
Disorders that Involve Anxiety Part 1

Generalized Anxiety Disorders
- Diagnostic Criteria, Prevalence, Contributing Factors, and Cultural Issues for Generalized Anxiety Disorder — Reading • 10 min
- Check Your Understanding: Generalized Anxiety Disorder — Practice Assignment • Grade: 100%

Panic Disorder
- Diagnostic Criteria, Prevalence, Contributing Factors, and Cultural Issues for Panic Disorder — Reading • 10 min
- Check Your Understanding: Panic Disorder — Practice Assignment • Grade: 80%

Specific Phobias
- Diagnostic Criteria and Prevalence for Specific Phobias — Reading • 15 min
- Contributing Factors for Specific Phobias — Reading • 10 min
- Classical Conditioning of Specific Phobias — Video • 2 min
- Operant Conditioning and Negative Reinforcement of Specific Phobias — Video • 3 min
- Cultural Issues for Specific Phobias — Reading • 2 min
- Check Your Understanding: Specific Phobias — Practice Assignment • Grade: 83.33%

Key Takeaways
- Key Takeaways: Disorders that Involve Anxiety Part 1 — Reading • 10 min

Module 5 Quiz
- Module Quiz: Disorders that Involve Anxiety Part 1 — Graded Assignment • Grade: 100%

Module 6
Disorders that Involve Anxiety Part 2

Social Anxiety Disorder
- **Diagnostic Criteria, Prevalence, Contributing Factors, and Cultural Issues for Social Anxiety Disorder** — Reading • 15 min
- Check Your Understanding: Social Anxiety Disorder — Practice Assignment • 5 min

Obsessive-Compulsive Disorder
- Diagnostic Criteria, Prevalence, Contributing Factors, and Cultural Issues for Obsessive-Compulsive Disorder — Reading • 15 min
- The Vicious Cycle of OCD — Video • 2 min
- Check Your Understanding: Obsessive-Compulsive Disorder — Practice Assignment • 4 min

Treatment for Disorders that Involve Anxiety
- Types of Treatment for Disorders that Involve Anxiety — Reading • 12 min
- Check Your Understanding: Treatment for Disorders that Involve Anxiety — Practice Assignment • 6 min

Key Takeaways
- Key Takeaways: Disorders that Involve Anxiety Part 2 — Reading • 10 min

Module 6 Quiz
- Module Quiz: Disorders that Involve Anxiety Part 2 — Graded Assignment • 7 min

Module 7
Conclusion

Module 8
Cumulative Quiz
- Cumulative Quiz: Anxiety and Related Disorders — Graded Assignment • 15 min

Module 9
Course Resources

Glossary: Psychology of Anxiety and Related Disorders
- Glossary — Reading • 15 min

References: Disorders that Involve Anxiety
- References — Reading • 2 min

Contributors: Disorders that Involve Anxiety
- Contributors — Reading • 2 min

Module 10
PsycLearn Essentials APA Student Resources

Diagnostic Criteria, Prevalence, Contributing Factors, and Cultural Issues for Social Anxiety Disorder

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Social Anxiety Disorder Example

Scenario

Malik walks up to the podium trying to calm down.

"Deep breath," Malik mutters to himself.

It isn't working. Malik feels his heart pounding in his chest and blood coursing through his temples. He starts to feel his face flush and he has the sensation of being hot and cold at the same time.

The class gives a muted applause. It's Malik's turn to present.

He didn't realize he was holding his breath, but then he heard himself gasp for air.

"Breathe man, breathe!" *he thinks.*

It's the same thing every time for Malik. He detests giving presentations. He'd much rather write a 20-page paper than speak in front of a classroom for 2 minutes.

At the podium and fumbles trying to connect his laptop to the projector.

"Deep breath."

He grabs the side of the podium. His knuckles slowly start turning white. He looks out at the classroom, above everybody's head. He avoids making eye contact, even with his friends. His classmates stare blankly at him. *"They probably think I'm a joke."*

Out of the corner of his eye, he notices his professor glance at him while making some notes on a writing pad.

"Oh, no. What have I done? I haven't even started yet!" Malik pauses.

"Deep breath."

Malik starts speaking and the muscles in his jaw lock up. Though he's practiced the presentation a dozen times, he can't remember any of what he was going to say. So, he ends up reading from his notes, sounding like a robot.

"Damn. I'm better than this."

It's going to be another painful presentation.

End of Scenario

Diagnostic Criteria

In and of itself, Malik's situation is far from unique. Perhaps you have experienced some of the same feelings. It is natural to feel a rush of anxiety before giving a presentation. Your sympathetic nervous system is working overtime, feeding your body adrenaline, raising your heart rate, and preparing you to take action. For most people, the anxiety that builds up before making their presentation tends to peak just before they speak and begins to dissipate once the presentation is under way.

For people with social anxiety disorder, on the other hand, the impairment they experience tends to be much greater. People with **social anxiety disorder** experience intense, persistent anxiety in social or performance situations that continues, often becoming more intense, the longer the situation lasts. This anxiety causes significant distress and will often lead people to avoid situations likely to create the anxiety. Social anxiety disorder tends to be exacerbated in situations where people feel they are being evaluated by others. Some common triggers for social anxiety include performing or talking in public, talking with an authority figure, meeting new people, going to parties, making small talk, eating in front of others, and using a public restroom. Situations in which the negative evaluation is more salient tend to create higher levels of distress. Further, for the experience of social anxiety to be considered disordered, it must be present and causing distress for a continued period of time, at least several months (WHO, 2019).

For people with social anxiety disorder, the anxiety tends to be more intense, longer lasting, and more disruptive than it is for most people. During these periods of heightened anxiety, people with social anxiety disorder become extremely aware of their physiological responses, such as increased heart rate, perspiration, and blushing. This awareness can lead people to have thoughts that make the anxiety worse. For example, someone may be thinking: (1) "I'm feeling anxious," (2) "Other people can see I'm feeling anxious," (3) "Other people are judging me," and (4) "People are thinking I'm awkward/I'm dumb/I should be fired." As you can imagine, having these thoughts running through your mind can be distracting enough to make doing anything else difficult, whether the person is trying to sing an aria on stage, answer a question in a meeting, or simply ask a stranger for directions.

Prevalence

The global lifetime prevalence of social anxiety disorder is approximately 4%. High-income countries like the United States, Japan, and Belgium have a higher prevalence (~5.5%) than lower-middle and low income countries like Colombia and Iraq (~1.6%). The United States has the highest lifetime prevalence of social anxiety disorder (~12%) and countries in Africa, the Eastern Mediterranean, and Eastern Europe have the lowest rates (~1.5%). Women are diagnosed with social anxiety disorder almost two times more frequently than men (Asher et al., 2017; Jefferies & Ungar, 2020).

Contributing Factors

Behavioral Factors

Just like specific phobias, social anxiety disorder can arise from personal experiences with a traumatic social interaction, such as being repeatedly berated for mistakes by a parent; or by vicarious experiences, such as watching other students get teased for a perceived failing (Mineka & Zinbarg, 2006).

Given the importance of social interaction in children's development, negative social interactions such as bullying and teasing are likely to predispose them to develop a social anxiety disorder later in life. For example, 92% of adults with social anxiety disorder reported experiencing severe teasing and bullying as children. On the other hand, only about 35% to 50% of other anxiety disorders were attributed to childhood bullying or teasing. This suggests that, while negative social interactions play a role in generating anxiety in general, they may be especially impactful on the development of social anxiety disorder (McCabe et al., 2003). Interestingly, the type of bullying matters, too. Individuals who suffer from social anxiety report experiencing greater levels of teasing related to physical appearance, sociability, and their performance when being evaluated (McCabe et al., 2010). Individuals who suffered from bullying in front of others, where the teasing involved a negative judgment of other children, are particularly at risk for developing social anxiety disorder. Notice how the contexts of bullying match up well with the triggers of social anxiety disorder discussed previously.

Cognitive Distortions

One effect of social anxiety disorder is the creation of **cognitive distortions**, which are inaccurate, typically negative, beliefs about your own abilities and what others think of you. People with social anxiety disorder often inaccurately assume that, whenever they are in the presence of other people, they are being evaluated. Because people with social anxiety are sensitive to being evaluated, this distortion intensifies any anxiety in a legitimately anxiety-provoking situation and creates anxiety where none need exist.

Some of the situations that generate social anxiety are actual, formal evaluations such as giving presentations or being interviewed for a job. Others are less formal, such as simply having a conversation with a group of people. In some cases, the perception that an audience is evaluating the person is the result of a cognitive distortion. That is, audience members are not really evaluating the person at all. In other cases, the audience itself is not real and the person is imagining what others would think of them if they were present (Heimberg et al., 2014). Whether these audience evaluations are real or imagined, individuals with social anxiety perceive and react to them as if they were real threats. People with social anxiety may experience the following cognitive distortions: distorted self-image, attentional bias, and interpretation bias.

Distorted Self-Image

Once a person suffering from social anxiety perceives the audience, they will form an internal representation of themselves. These representations of the self are often distorted. For example, when asked to give a speech, individuals with high social anxiety have more negative mental images of themselves than people without the disorder. These negative perceptions can be seen with respect to their social competence ("I am boring"), signs of their anxiety ("I am stuttering"), and their physical appearance ("I am unattractive"). The negative self-images generated from these distortions are associated with greater levels of anxiety and a greater degree of overestimating the negativity of others' perceptions of them (Chiupka et al., 2012; Heimberg et al., 2014).

Attentional Bias

Individuals with social anxiety disorder often perceive other people as threats. This will lead them to avoid social situations or, if that isn't possible, to distract themselves from the other people nearby. In a lab setting, Yiping Chen and colleagues (2002) demonstrated that, when shown pictures of faces and household items, people with social anxiety focused more on the household items than the faces, whereas people without social anxiety focused equally on each picture category. The participants with social anxiety showed an attentional bias against the faces. This avoidance strategy is likely helpful in managing anxiety, but it also means that people with social anxiety aren't able to pick up cues from the environment. Such cues might suggest that people aren't evaluating them or that people are evaluating them positively.

Interpretation Bias

Finally, individuals with elevated levels of social anxiety hold distorted perceptions that lead them to interpret neutral stimuli as threatening. For example, in one study, socially anxious participants who received positive, neutral, and negative feedback perceived the neutral feedback (e.g., "That's an interesting shirt you're wearing") more negatively than did the other participants, who either had no anxiety or whose anxiety was not socially based (Amir et al., 2005).

Overall, cognitive distortions lead socially anxious people to perceive the social world as more threatening than it really is. As a result, they perceive themselves negatively and avoid social situations. Not surprisingly, people with social anxiety have difficulty forming relationships, including romantic relationships, as a result of these distortions (Sparrevohn & Rapee, 2009); and this further distorts their perceptions of how others evaluate them (Rapee & Heimberg, 1997).

Cultural Issues

Cultural influences on acting in public vary widely. In some cultures, there is a strong expectation that individuals will engage in public behavior such as speaking, singing, or dancing. In such cultures, many people may feel some anxiety at these expectations. The anxiety experienced is only considered disordered when it is outside the norms for the society and causes some degree of distress. In other cultures, the norm is to not engage in public behavior such as speaking or expressing one's opinions. In this type of cultural context, the avoidance of social behavior will not be considered symptomatic of social anxiety disorder unless the level of anxiety violates societal expectations.

The experience of social anxiety and the distress it causes is unique to a particular culture. For example, taijin kyofusho is a culture-specific variation of social anxiety disorder that is primarily seen in Japan. Its primary characteristic is that individuals with the disorder have an intense fear of offending others, as opposed to a fear of being evaluated negatively (Tseng et al., 2012). Individuals with taijin kyofusho fear exhibiting socially inappropriate behaviors, such as excessive eye contact or personal defects such as unpleasant body odor because others may find those things offensive and be put into a negative state as a result. This focus on interpersonal relationships and the effects of the self on other people distinguishes taijin kyofusho apart from the social anxiety typically seen in Western countries, which typically focuses on anxiety as a result of the threat of negative evaluations of the self.

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